

Multidisciplinary Stroke Unit Schedule

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Certification Alignment Note: Neurointerventional availability must match the actual approved roster. Where 24/7 on-site thrombectomy coverage is not available, the documented backup/transfer pathway must be activated and evidence retained.

Roles, Responsibilities & Meeting Cadence

Prepared in Support of American Stroke Association International Comprehensive Stroke Center Certification

This document defines the composition, daily/weekly/monthly operating rhythm, and individual accountabilities of the stroke multidisciplinary team (MDT), aligned to American Stroke Association International Stroke Center Certification program / American Heart Association Comprehensive Stroke Center requirements. It is intended to guide day-to-day operations and to serve as evidence of a structured, functioning multidisciplinary program during survey.

1. Core Multidisciplinary Team — Roles & Responsibilities

Stroke Medical Director (Vascular Neurologist)

- Overall clinical and administrative oversight of the stroke program
- Approves and reviews protocols/order sets annually
- Chairs the Stroke Committee
- Reviews all mortality, complication, and deviation cases
- Serves as physician champion for GWTG-Stroke data quality

Neurointerventionalist

- availability per approved neurointerventional roster, with a defined backup/transfer pathway for coverage gaps
- Performs and documents endovascular procedures, TICI scoring
- Participates in door-to-groin performance review

Neurosurgeon

- 24/7 availability for hemorrhagic stroke, decompressive craniectomy, EVD placement
- Co-manages ICH/SAH patients with neurocritical care

Neurocritical Care Intensivist

- Manages ICU admissions post-thrombolysis/thrombectomy and hemorrhagic stroke
- Directs ICP/BP management protocols
- Leads daily ICU multidisciplinary rounds

Emergency Medicine Physician

- Activates stroke code / stroke alert on arrival
- Completes NIHSS and initial assessment within target time
- Coordinates door-to-needle workflow with stroke team

Stroke Program Coordinator (RN, BSN/MSN)

- Program's day-to-day operational lead
- Oversees concurrent and retrospective chart audits
- Coordinates staff education, drills, and competency validation
- Liaises with EMS and referring facilities
- Prepares data/reports for Stroke Committee and site visit readiness

Stroke Unit Charge Nurse / Staff RN

- Performs serial NIHSS, neuro checks, VTE/skin/fall risk assessments
- Executes stroke order sets and monitors thrombolysis infusions
- Documents timestamps for core measures (door-to-CT, door-to-needle)

Radiology / CT-MRI Technologist

- Prioritizes stroke-code imaging ("CT ahead" protocol)
- Ensures timely CT/CTA/MRI acquisition and image transfer to reading radiologist/teleradiology

Interventional Radiology Technologist

- Prepares angio suite for emergent thrombectomy
- Supports procedural documentation of recanalization times

Clinical Pharmacist (Stroke-specialized)

- Verifies alteplase/tenecteplase dosing and contraindications
- Reviews anticoagulant/antiplatelet regimens and reconciles discharge medications
- Participates in P&T review of stroke-related medications

Physical Therapist (PT)

- Completes early mobility and functional assessment within 24–48 hours
- Develops rehabilitation goals and mobility plan

Occupational Therapist (OT)

- Assesses ADL function and cognitive-perceptual deficits
- Provides adaptive equipment recommendations

Speech-Language Pathologist (SLP)

- Performs bedside dysphagia screening prior to oral intake
- Assesses speech, language, and cognitive-communication deficits

Case Manager / Social Worker

- Coordinates discharge disposition (home, acute rehab, SNF)
- Addresses psychosocial needs, insurance authorization, caregiver readiness

Registered Dietitian

- Assesses nutritional status and swallow-safe diet needs post-SLP clearance
- Provides secondary-prevention diet education

Quality / PI Data Abstractor

- Abstracts and submits GWTG-Stroke registry data
- Tracks core/quality measure compliance and flags outliers
- Prepares performance dashboards for committee review

Laboratory Services

- Ensures rapid turnaround of stroke-code labs (glucose, coagulation, CBC)
- Maintains point-of-care testing quality controls

Research Coordinator

- Screens/enrolls eligible patients in stroke clinical trials (CSC requirement)
- Maintains IRB documentation and research program metrics

Rehabilitation Medicine Physician (Physiatrist)

- Evaluates inpatient rehab candidacy
- Determines mRS/functional trajectory and rehab level of care

EMS Liaison

- Coordinates pre-notification protocols with regional EMS
- Provides feedback on pre-hospital LKW/FAST documentation quality

2. Daily Operating Schedule

Applies seven days a week; stroke code response is 24/7/365 regardless of the schedule below.

Time	Activity	Responsible / Attendees	Purpose
06:30–07:00	Overnight event handoff / stroke code review	Charge RN, Night Hospitalist/Intensivist	Review any overnight stroke activations, code compliance, and outstanding tasks
07:30–08:30	ICU multidisciplinary rounds	Intensivist, Neurology, Nursing, Pharmacy, PT/OT	Daily plan of care, sedation/BP goals, extubation/mobility readiness
09:00–09:30	Stroke unit multidisciplinary huddle	Coordinator, Bedside RN, PT/OT/SLP, Case Manager, Dietitian	Review each patient's NIHSS trend, therapy needs, discharge barriers
09:30–12:00	Rehabilitation assessments (PT/OT/SLP)	PT, OT, SLP	New assessments within 24–48h of admission; ongoing therapy sessions
10:00–11:00	Physician/Neurology rounds	Attending Neurologist, Residents/APPs	Neuro exam, NIHSS reassessment, secondary-prevention workup review
Continuous	Stroke code coverage	ED Physician, Neurology on-call, Stroke RN, CT Tech, Pharmacy, Lab	Door-to-CT ≤ 20 min, Door-to-Needle ≤ 60 min, Door-to-Groin per protocol
13:00–13:30	Case management / discharge planning huddle	Case Manager, Social Worker, Coordinator	Disposition planning, insurance authorization, caregiver education needs
15:00–15:30	Chart/documentation spot-check	Stroke Coordinator	Concurrent review of timestamps, NIHSS documentation, core-measure fields
19:00–19:30	Evening nursing handoff	Outgoing/Incoming Charge RN	Neuro status, pending swallow/therapy orders, VTE/skin reassessment due

3. Weekly Operating Schedule

Day	Activity	Lead / Attendees	Purpose
Monday	Multidisciplinary stroke rounds (all active inpatients)	Full MDT	Comprehensive case-by-case review of active stroke patients and rehab trajectory
Tuesday	Stroke order set / protocol compliance audit	Stroke Coordinator, Pharmacist	Review a sample of charts against current protocols
Wednesday	Neurointerventional / thrombectomy case review	Neurointerventionalist, Neurology, Coordinator	Review door-to-groin and recanalization times, complications
Wednesday	Staff education in-service	Coordinator, unit educators	NIHSS competency, new protocol rollouts, simulation debriefs
Thursday	Mortality & morbidity (M&M) case review	Medical Director, Neurosurgery, Intensivist	In-depth review of any deaths, hemorrhagic conversions, or complications
Friday	GWTG-Stroke data quality review	Data Abstractor, Coordinator	Reconcile abstracted cases, resolve data queries before registry lock
Friday	Mock stroke code / simulation drill (rotating)	ED, Nursing, Radiology, Pharmacy	Timed simulation to sustain readiness and identify workflow gaps

4. Monthly & Quarterly Schedule

Frequency	Activity	Lead / Attendees	Purpose
Monthly	Stroke Program Committee meeting	Medical Director (chair), full MDT leadership	Review performance dashboard, protocol updates, action-item tracking
Monthly	Performance & Quality (PI) data review	Data Abstractor, Medical Director, Coordinator	Core measure trends, outlier case review, benchmark comparison
Monthly	EMS / pre-hospital feedback session	EMS Liaison, Coordinator, ED Medical Director	Pre-notification accuracy, LKW documentation, transport time trends
Quarterly	Peer review of thrombolysis/thrombectomy cases	Medical Director, Neurointerventionalist, Neurosurgery	Formal peer review per CSC requirement
Quarterly	Community & professional education outreach	Coordinator, Research Coordinator	Public stroke-awareness events, CME/CE sessions (CSC outreach requirement)
Quarterly	Research program review	Research Coordinator, Medical Director	Enrollment metrics, active protocol status, IRB compliance
Annually	Protocol / order set revision and staff competency re-validation	Medical Director, Coordinator, Pharmacy	Update protocols to current AHA/ASA guidelines; re-certify NIHSS competency
Annually	Full mock survey / readiness assessment	Full MDT	Simulated certification survey with document and tracer review

5. AHA Comprehensive Stroke Center Survey Preparation Timeline

6 months out

- Medical Director: confirm all protocols reflect current AHA/ASA guideline updates
- Coordinator: begin comprehensive chart audit against certification standards
- Data Abstractor: verify 12 months of GWTG-Stroke data completeness and accuracy
- Research Coordinator: confirm active trial enrollment meets CSC volume requirement

3 months out

- Coordinator: schedule mock survey / tracer methodology drill
- All disciplines: complete annual competency validations (NIHSS, TICI, dysphagia screening)
- Pharmacy: audit thrombolytic dosing and time-to-administration documentation
- Case Management: verify discharge/education documentation completeness

1 month out

- Medical Director & Coordinator: finalize performance improvement project documentation
- All team leads: assemble evidence binders (policies, meeting minutes, education logs, drill records)
- Nursing leadership: reinforce staff readiness for surveyor interviews

Survey week

- Coordinator: serve as primary surveyor liaison and document custodian
- Medical Director: available for physician-level interviews and case walkthroughs
- All MDT members: available for role-specific tracer interviews on request
- Unit leadership: ensure real-time chart/documentation accuracy on all active patients